Proposed Prescribing Information
For the use of a Registered Medical Practitioner or a Hospital or a Laboratory

KARVIL TABLETS 6.25 MG AND
KARVIL TABLETS 25 MG
CARVEDILOL TABLETS USP 6.25 MG AND 25 MG

BRAND OR PRODUCT NAME
KARVIL TABLETS 6.25 MG
KARVIL TABLETS 25 MG

NAME AND STRENGTH OF ACTIVE SUBSTANCE(S)

KARVIL TABLETS 6.25 MG
Each uncoated tablet contains:
Carvedilol……...6.25mg

KARVIL TABLETS 25 MG
Each uncoated tablet contains:
Carvedilol……...25mg

PRODUCT DESCRIPTION

KARVIL TABLETS 6.25 MG
White to off-white colored, round, flat, uncoated tablets with break line on one side and plain on other side.

KARVIL TABLETS 25 MG
White to off-white colored, round, flat, uncoated tablets with break line on one side and plain on other side.

DOSAGE FORM
Uncoated Tablet

PHARMACODYNAMICS / PHARMACOKINETICS
Pharmacodynamics
Carvedilol is a multiple action adrenergic receptor blocker with ά1, β1 and β2 adrenergic receptor blockade properties. Carvedilol has been shown to have organ-protective effects. Carvedilol is a potent antioxidant and a scavenger of reactive oxygen radicals. Carvedilol is racemic, and both R(+) and S(-) enantiomers have the same s-adrenergic receptor blocking properties and antioxidant properties. Carvedilol has antiproliferative effects on human vascular smooth muscle cells. A decrease in oxidative stress has been reported in clinical studies by measuring various markers during chronic treatment of patients with carvedilol. Carvedilol's β-adrenergic receptor blocking properties are nonselective for the β1 and β2-adrenoceptors and are associated with the levorotatory S(-) enantiomer Carvedilol has no intrinsic sympathomimetic activity and (like propranolol) it has membrane stabilising

properties. Carvedilol suppresses the renin-angio-tensin-aldosterone system through β-blockade, which reduces the release of renin, thus making fluid retention rare.
Carvedilol reduces the peripheral vascular resistance via selective blockade of ά1-adrenoceptors. Carvedilol attenuates the increase in blood pressure induced by phenylephrine, an ά1- adrenoceptor agonist, but not that induced by angiotensin II. Carvedilol has no adverse effect on the lipid profile. A normal ratio of high-density lipoproteins to low density lipoproteins (HDL/LDL) is maintained.

Hypertension
Carvedilol lowers blood pressure in hypertensive patients by a combination of β-blockade and ά1 mediated vasodilation. A reduction in blood pressure is not associated with a concomitant increase in total peripheral resistance, as observed with pure β-blocking agents. Heart rate is slightly decreased. Renal blood flow and renal function are maintained in hypertensive patients. Carvedilol has been reported to maintain stroke volume and reduce total peripheral resistance. Blood supply to distinct organs and vascular beds including kidneys, skeletal muscles, forearms, legs, skin, brain or the carotid artery is not compromised by carvedilol. There is a reduced incidence of cold extremities and early fatigue during physical activity. The long-term effect of carvedilol on hypertension is documented in several double-blind controlled studies.

Coronary heart disease
In patients with coronary heart disease, carvedilol has demonstrated anti-ischemic (improved total exercise time, time to 1 mm ST segment depression and time to angina) and antianginal properties that were maintained during long-term treatment.

Chronic heart failure
Carvedilol significantly reduces all cause mortality and the need for cardiovascular hospitalization. Carvedilol also increases ejection fraction and improves symptoms in patients with ischemic or non-ischemic chronic heart failure. The effect of carvedilol is dose dependent.

Pharmacokinetics
Carvedilol is well absorbed from the gastrointestinal tract but is subject to considerable first-pass metabolism in the liver; the absolute bioavailability is about 25%. Peak plasma concentrations occur 1 to 2 hours after an oral dose. It has high lipid solubility. Carvedilol is more than 98% bound to plasma proteins. It is extensively metabolised in the liver, primarily by the cytochrome P450 isoenzymes CYP2D6 and CYP2C9, and the metabolites are excreted mainly in the bile. The elimination half-life is about 6 to 10 hours.

<u>Pharmacokinetics in special populations</u>
Patients with renal impairment
The autoregulatory blood supply is preserved and the glomerular filtration is unchanged during chronic treatment with carvedilol.

In patients with hypertension and renal insufficiency, the area under plasma level-time curve, elimination half-life and maximum plasma concentration does not change significantly. Renal excretion of the unchanged drug decreases in the patients with renal insufficiency; however, changes in pharmacokinetic parameters are modest.

Several open studies have reported that carvedilol is an effective agent in patients with renal hypertension. The same is true in patients with chronic renal failure or those on hemodialysis or after renal transplantation. Carvedilol causes a gradual reduction in blood pressure both on dialysis and non-dialysis days, and the blood pressure-lowering effects are comparable with those seen in patients with normal renal function. Carvedilol is not eliminated during dialysis because it does not cross the dialysis membrane, probably due to its high plasma protein binding.

Patients with hepatic impairment

In patients with cirrhosis of the liver, the systemic availability of the drug is increased by up to 80% because of a reduction in the first-pass effect. Therefore, carvedilol is contraindicated in patients with clinically manifest liver dysfunction.

Geriatric use

The pharmacokinetics of carvedilol in hypertensive patients were not affected by age. A study in elderly hypertensive patients reported that there was no difference in the adverse event profile. Another study which included elderly patients with coronary heart disease reported no difference in the adverse events.

Pediatric use

There is limited data available on pharmacokinetics in people younger than 18 years of age.

Diabetic patients

In hypertensive patients with non-insulin-dependent diabetes no influence of carvedilol on fasting or post-prandial blood glucose concentration, glycolated hemoglobin A_1 or need for change of the dose of antidiabetic agents was demonstrated. In patients with non-insulin dependent diabetes, carvedilol had no statistically significant influence on the glucose tolerance test. In hypertensive non-diabetic patients with impaired insulin sensitivity (syndrome X) carvedilol improved the insulin sensitivity. The same results were found in hypertensive patients with non-insulin dependent diabetes.

INDICATION

Hypertension

Carvedilol is indicated primarily for the management of essential hypertension. It can be used alone or in combination with other antihypertensive agents (e. g. calcium channel blockers, diuretics).

Treatment of angina pectoris

Treatment of symptomatic chronic heart failure

Carvedilol is indicated for the treatment of symptomatic chronic heart failure (CHF) to reduce mortality and cardiovascular hospitalisations, improve patient well-being and slow the progression of the disease.

Carvedilol may be used as adjunct to standard therapy, but may also be used in those patients unable to tolerate an ACE inhibitor, or those who are not receiving digitalis, hydralazine or nitrate therapy.

RECOMMENDED DOSAGE AND MODE OF ADMINISTRATION

Duration of treatment

Treatment with carvedilol is a long-term therapy. Treatment should not be stopped abruptly but rather gradually reduced at weekly intervals. This is particularly important in the case of patients with concomitant coronary heart disease.

Essential hypertension

The recommended dose for initiation of therapy is 12.5 mg once a day for the first 2 days. Thereafter, the recommended dosage is 25 mg once a day. If necessary, the dosage may subsequently be increased at intervals of at least two weeks to the recommended maximum daily dose of 50 mg given once daily or in divided doses (twice daily).

Angina pectoris

The recommended dose for initiation of therapy is 12.5 mg twice a day for the first 2 days. Thereafter the recommended dosage is 25 mg twice a day. If necessary, the dosage may there after be increased at intervals of at least two weeks, up to the recommended maximum daily dose of 100 mg given in divided doses (twice daily).

Symptomatic, stable, chronic heart failure

Dosage must be tailored to suit the individual and closely monitored by a physician during up-titration.

For those patients receiving digitalis, diuretics and ACE inhibitors, dosing of these drugs should be stabilized before initiation of carvedilol treatment.

The recommended dose for initiation of therapy is 3.125 mg twice daily for 2 weeks. If this dose is tolerated, the dose may thereafter be increased, at intervals of not less than two weeks to 6.25 mg, 12.5 mg and 25 mg twice daily. Doses should be increased to the highest level tolerated by the patient.

The maximum recommended dose is 25 mg twice daily for all patients with severe CHF and for patients with mild to moderate CHF weighing less than 85 kg (187 lbs). In patients with mild or moderate CHF weighing more than 85 kg, the maximum recommended dose is 50 mg twice daily.
Before each dose increase, the patient should be evaluated by the physician for symptoms of vasodilation or worsening heart failure.

Transient worsening of heart failure or fluid retention should be treated with increased doses of diuretics. Occasionally, it may be necessary to lower the dose of carvedilol and, in rare cases, temporarily discontinue carvedilol treatment.

If carvedilol treatment is discontinued for more than one week, therapy should be recommenced at a lower dose level (twice daily) and up-titrated in line with the above dosing recommendation. If carvedilol is discontinued for more than two weeks, therapy should be recommenced at 3.125 mg in line with the above dosing recommendation. Symptoms of vasodilation may be managed initially by a reduction in the dose of diuretics. If symptoms persist, the dose of ACE inhibitor (if used) may be reduced, followed by a reduction in the dose of carvedilol if necessary. Under these circumstances, the dose of carvedilol should not be increased until symptoms of worsening heart failure or vasodilation have been stabilized.

Special dosage instructions
Renal impairment
Available pharmacokinetic data in patients with varying degrees of renal impairment (including renal failure) suggest no changes in carvedilol dosing recommendations are warranted in patients with moderate to severe renal insufficiency.

Hepatic impairment
Carvedilol is contraindicated in patients with clinical manifestations of liver dysfunction.

Elderly
There is no evidence to support dose adjustment.

Children
Safety and efficacy in children (under 18 years) has not been established.

Mode of Administration
The tablets are to be swallowed with sufficient fluid.

CONTRAINDICATIONS
Carvedilol must not be used in patients with:
- Hypersensitivity to carvedilol or any component of the product
- Unstable/decompensated heart failure
- Clinically manifest liver dysfunction

As with other B-blockers, carvedilol must not be used in patients with:
- 2nd and 3rd degree AV block (unless a permanent pacemaker is in place)
- Severe bradycardia (< 50 bpm)
- Sick sinus syndrome (including sino-atrial block)
- Severe hypotension (systolic blood pressure < 85 mmHg)
- Cardiogenic shock
- History of bronchospasm or asthma

WARNINGS AND PRECAUTIONS
Cessation of Therapy
Patients with coronary artery disease, who are being treated with carvedilol, should be advised against abrupt discontinuation of therapy. Severe exacerbation of angina and the occurrence of myocardial infarction and ventricular arrhythmias have been reported in

angina patients following the abrupt discontinuation of therapy with β-blockers. The last 2 complications may occur with or without preceding exacerbation of the angina pectoris. As with other β-blockers, when discontinuation of carvedilol is planned, the patients should be carefully observed and advised to limit physical activity to a minimum. Carvedilol should be discontinued over 1 to 2 weeks whenever possible. If the angina worsens or acute coronary insufficiency develops, it is recommended that carvedilol be promptly reinstituted, at least temporarily. Because coronary artery disease is common and may be unrecognized, it may be prudent not to discontinue therapy with carvedilol abruptly even in patients treated only for hypertension or heart failure.

Chronic heart failure

In chronic heart failure patients, worsening cardiac failure or fluid retention may occur during up-titration of carvedilol. If such symptoms occur, diuretics should be increased and the carvedilol dose should not be advanced until clinical stability resumes. Occasionally, it may be necessary to lower the carvedilol dose or, in rare cases, temporarily discontinue it. Such episodes do not preclude subsequent successful titration of carvedilol. Carvedilol should be used with caution in combination with digitalis glycosides, as both drugs slow AV conduction.

Renal function in chronic heart failure

Reversible deterioration of renal function has been reported with carvedilol therapy in chronic heart failure patients with low blood pressure (systolic BP <100 mmHg), ischemic heart disease and diffuse vascular disease, and-/or underlying renal insufficiency.

Chronic obstructive pulmonary disease

Carvedilol should be used with caution, in patients with chronic obstructive pulmonary disease (COPD) with a bronchospastic component who are not receiving oral or inhaled medication, and only if the potential benefit outweighs the potential risk. In patients with a tendency to bronchospasm, respiratory distress can occur as a result of a possible increase in airway resistance. Patients should be closely monitored during initiation and up-titration of carvedilol and the dose of carvedilol should be reduced if any evidence of bronchospasm is observed during treatment.

Non-allergic Bronchospasm

Patients with bronchospastic disease (e.g., chronic bronchitis and emphysema) should, in general, not receive b-blockers. Carvedilol may be used with caution, however, in patients who do not respond to, or cannot tolerate, other antihypertensive agents. If carvedilol is used, the smallest effective dose should be used, so that inhibition of endogenous or exogenous β-agonists is minimized.

Diabetes

Care should be taken in the administration of carvedilol to patients with diabetes mellitus, as the early signs and symptoms of acute hypoglycemia may be masked or attenuated. In chronic heart failure patients with diabetes, the use of carvedilol may be associated with worsening control of blood glucose.

Peripheral Vascular disease
Carvedilol should be used with caution in patients with peripheral vascular disease as β-blockers can precipitate or aggravate symptoms of arterial insufficiency.

Raynaud's phenomenon
Carvedilol should be used with caution in patients suffering from peripheral circulatory disorders (e. g. Raynaud's phenomenon) as there may be exacerbation of symptoms.

Thyrotoxicosis
Carvedilol, like other agents with β-blocking properties, may obscure the symptoms of thyotoxicosis.

Anesthesia and major surgery
Caution should be exercised in patients undergoing general surgery, because of the synergistic negative ionotropic effects of carvedilol and anesthetic drugs.

Intraoperative Floppy Iris Syndrome
Intraoperative Floppy Iris Syndrome (IFIS) has been reported during cataract surgery in some patients treated with alpha-1 blockers. This variant of small pupil syndrome is characterized by the combination of a flaccid iris that billows in response to intraoperative irrigation currents, progressive intraoperative miosis despite preoperative dilation with standard mydriatic drugs, and potential prolapse of the iris toward the phacoemulsification incisions. The patient’s ophthalmologist should be prepared for possible modifications to the surgical technique, such as utilization of iris hooks, iris dilator rings, or viscoelastic substances. There does not appear to be a benefit of stopping alpha-1 blocker therapy prior to cataract surgery.

Bradycardia
Carvedilol may induce bradycardia. If the patient's pulse rate decreases to less than 55 beats per minute, the dosage of carvedilol should be reduced.

Hypersensitivity
Care should be taken in administering carvedilol to patients with a history of serious hypersensitivity reactions, and if those undergoing desensitisation therapy, as β-blockers may increase both the sensitivity towards allergens and the seriousness of anaphylactic reactions.

Psoriasis
Patients with a history of psoriasis associated with β-blocker therapy should take carvedilol only after consideration of the risk-benefit ratio.

Concomitant use of calcium channel blockers
Careful monitoring of ECG and blood pressure is necessary in patients receiving concomitant therapy with calcium channel blockers of the verapamil or diltiazem type or other antiarrhythmic drugs.

Pheochromocytoma
In patients with pheochromocytoma, ά-blocking agent should be initiated prior to the use of any β-blocking agent. Although carvedilol has both ά- and β-blocking pharmacological activities, there is no experience with its use in this condition. Caution should therefore be taken in the administration of carvedilol to patients suspected of having pheochromocytoma.

Prinzmetal's variarut angina
Agents with non-selective β-blocking activity may provoke chest pain in patients with Prinzmetal's variant angina. There is no clinical experience with carvedilol in these patients although the ά-blocking activity of carvedilol may prevent such symptoms. Caution should; however, be taken in the administration of carvedilol to patients suspected of having Prinzmetal's variant angina.

Contact lenses
Wearers of contact lenses should bear in mind the possibility of reduced lacrimation.

Fitness to drive
No studies have been reported on the effects of carvedilol on patients' fitness to drive or to operate machinery. Because of individually variable reactions (e. g. dizziness, tiredness), the ability to drive, operate machinery, or work without firm support may be impaired. This applies particularly at the start of treatment, after dose increases, on changing products, and in combination with alcohol.

Children: safety and efficacy have not been established.

INTERACTIONS WITH OTHER MEDICAMENTS
Pharmacokinetic interactions
Digoxin: Digoxin concentrations are increased by about 15%, when digoxin and carvedilol are administered concomitantly. Both digoxin and carvedilol slow AV conduction. Increased monitoring of digoxin levels is recommended when initiating, adjusting or discontinuing carvedilol.

Insulin or oral hypoglycemics: Agents with β-blocking properties may enhance the blood-sugar-reducing effect of insulin and oral hypoglycemics. The signs of hypoglycemia may be masked or attenuated (especially tachycardia). In patients taking insulin or oral hypoglycemics, regular monitoring of blood glucose is therefore recommended.

lnducers and inhibitors of hepatic metabolism: Rifampicin reduced plasma concentrations of carvedilol by about 70%. Cimetidine increased AUC by about 30% but caused no change in C_{max}. Care may be required in those patients receiving inducers of mixed function oxidases e. g. rifampicin, as serum levels of carvedilol may be reduced, or inhibitors of mixed function oxidases e.g. cimetidine, as serum levels of carvedilol may be increased. However, based on the relatively small effect of cimetidine on carvedilol drug levels, the likelihood of any clinically important interaction is minimal.

Catecholamine-depleting agents: Patients taking both agents with β-blocking properties and a drug that can deplete catecholamines (e.g. reserpine and monoamine oxidase inhibitors) should be observed closely for signs of hypotension and/or severe bradycardia.

Cyclosporin: Modest increases in mean trough cyclosporin concentrations were reported following initiation of carvedilol treatment in 21 renal transplant patients suffering from chronic vascular rejection. In about 30% of patients the dose of cyclosporin had to be reduced in order to maintain cyclosporin concentrations within the therapeutic range, while in the remainder no adjustment was needed. On average, the dose of cyclosporin was reduced about 20% in these patients. Due to wide lnter-individual variability in the dose adjustment required, it is recommended that cyclosporin concentrations be monitored closely after initiation of carvedilol therapy and that the dose of cyclosporin be adjusted as appropriate.

Verapamil, diltiazem, or other antiarrhythmic: In combination with carvedilol can increase the risk of AV conduction disturbances.

Amiodarone

Amiodarone, and its metabolite desethyl amiodarone, inhibitors of CYP2C9 and P-glycoprotein, increased concentrations of the S(-)- enantiomer of carvedilol by at least 2-fold. The concomitant administration of amiodarone or other CYP2C9 inhibitors such as fluconazole with carvedilol may enhance the β-blocking properties of carvedilol resulting in further slowing of the heart rate or cardiac conduction. Patients should be observed for signs of bradycardia or heart block, particularly when one agent is added to pre-existing treatment with the other.

Pharmacodynamic interactions

Clonidine: Concomitant administration of clonidine with agents with β-blocking properties may potentiate blood-pressure- and heart-rate-lowering effects. When concomitant treatment with agents with β-blocking properties and clonidine is to be terminated, the β-blocking agent should be discontinued first. Clonidine therapy can then be discontinued several days later by gradually decreasing the dosage.

Calcium channel blockers: Isolated cases of conduction disturbance (rarely with hemodynamic compromise) have been reported when carvedilol is coadministered with diltiazem. As with other agents with β-blocking properties, if carvedilol is to be administered orally with calcium channel blockers of the verapamil or diltiazem type, it is recommended that ECG and blood pressure be monitored. As with other agents with β-blocking activity, carvedilol may potentiate the effect of other concomitantly administered drugs that are anti-hypertensive in action (e.g. ά1-receptor antagonists) or have hypotension as part of their adverse effect profile. Careful attention must be paid during anesthesia to the synergistic negative ionotropic and hypotensive effects of carvedilol and anesthetic drugs.

Certain sedatives (barbiturates, phenothiazines) and drugs for treating depression (tricyclic antidepressant), vasodilating drugs and alcohol may intensify the blood pressure lowering effect.

STATEMENT ON USAGE DURING PREGNANCY AND LACTATION

Pregnancy

Beta-blockers reduce placental perfusion, which may result in intrauterine fetal death, and immature and premature deliveries. In addition, adverse effects (especially hypoglycemia and bradycardia) may occur in the fetus and neonate. There may be an increased risk of cardiac and pulmonary complications in the neonate in the postnatal period. There is no evidence from animal studies that carvedilol has any teratogenic effects. There is no adequate clinical experience with carvedilol in pregnant women. Carvedilol should not be used during pregnancy unless the potential benefit outweighs the potential risk.

Lactation

Animal studies demonstrated that carvedilol or its metabolites are excreted in breast milk. It is not known whether carvedilol is excreted in human milk. Breast-feeding is therefore not recommended during administration of carvedilol.

ADVERSE EFFECTS / UNDESIRABLE EFFECTS

The frequency of adverse experiences is not dose dependent, with the exception of dizziness, abnormal vision and bradycardia.

Undesirable effects in chronic heart failure

Adverse experiences most frequently observed in the carvedilol group in clinical trials in congestive heart failure patients and not seen at an equivalent incidence among placebo treated patients are described below,

Central nervous System

Very common: dizziness, headaches are usually mild and occur particularly at the start of treatment. Asthenia - (including fatigue) also occurs very commonly.

Cardiovascular system

Common: bradycardia, postural hypotension, hypotension, edema (including generalized, peripheral, dependent and genital edema, edema of the legs, hypervolemia and fluid overload).

Uncommon: syncope (including presyncope), AV-block and cardiac failure during up-titration.

Gastro-intestinal system

Commonly nausea, diarrhea and vomiting.

Hematology

Rare: thrombocytopenia

Leucopenia has been reported in isolated cases

Metabolic:
Commonly: Weight increase and hypercholesterolemia.
Hyperglycemia, hypoglycemia and worsening control of blood glucose are also common in patients with pre-existing diabetes mellitus

Others
Commonly: vision abnormalities
Rarely: renal failure and renal function abnormalities in patients with diffuse vascular disease and/or impaired renal function

Undesirable effects in hypertension and the long term management of coronary heart disease
The profile of adverse events associated with the use of carvedilol in the treatment of hypertension and the long term management of coronary heart disease is consistent with that observed in chronic heart failure. The incidence of adverse events in these patient populations is lower, however. Adverse experiences reported in clinical trials in patients with hypertension and coronary heart diseases are:

Central nervous system
Common: dizziness, headaches and fatigue, which are usually mild and occur particularly at the beginning of treatment.
Uncommon: depressed mood, sleep disturbance, paresthesia.

Cardiovascular system
Common: bradycardia, postural hypotension and uncommonly syncope, especially at the beginning of treatment.

Uncommon: disturbance of peripheral circulation (cold extremities, PVD, exacerbation of intermittent claudication and Raynauds phenomenon), AV-block, angina pectoris (including chest pain), symptoms of heart failure and peripheral edema.

Respiratory system
Common: Asthma and dyspnea in predisposed patients.
Rare: Stuffy nose.

Gastro-intestinal system
Common: gastro-intestinal upset (with symptoms such as nausea, abdominal pain, diarrhea).
Uncommon: constipation and vomiting.

Skin and appendages
Uncommon: skin reactions (eg. allergic exanthema, dermatitis, urticaria and pruritis).

Blood chemistry and hematology
Isolated cases of increases in ALAT and gamma GT, thrombocytopenia and leucopenia have been reported.

Others
Common: pain in extremities.
Common: reduced lacrimation and eye irritation.
Uncommon: cases of sexual impotence disturbed vision.
Rare: dryness of mouth and disturbances of micturition.
Isolated cases of allergic reactions have been reported.

OVERDOSE AND TREATMENT
Symptoms and signs of intoxication
In the event of overdosage, there may be severe hypotension, bradycardia, heart failure, cardiogenic shock and cardiac arrest. There may also be respiratory problems, bronchospasm, vomiting, disturbed consciousness and generalised seizures. Treatment of intoxication, In addition to general procedures, the vital parameters must be monitored and corrected, if necessary, under intensive care conditions.

The following supportive therapies can be used:
Patients should be placed in the supine position. Atropine: 0.5 to 2 mg i.v (for excessive bradycardia). Glucagon: initially 1 to 10 mg i.v then 2 to 5 mg/h as a long term infusion (to support cardiovascular function). Sympathonimetics according to body-weight and effect: dobutamine, isoprenaline, orciprenaline or adrenaline. If positive ionotropic effect is required, phosphodiesterase inhibitors; (pDE) e.g. milrinone should be considered. If peripheral vasodilation dominates the intoxication profile then norfenephrine or noradrenaline should be administered with continuous monitoring of the circulatory conditions. In the case of drug-resistant bradycardia, pacemaker therapy should be initiated. Treatment of bronchospasm: For bronchospasm, β-Sympathomimetics (as aerosol or i.v) or aminophylline i.v. should be given.

Treatment of seizures In the event of seizures, slow i.v injection of diazepam or clonazepam is recommended.

Important note in cases of severe intoxication with shock, supportive treatment must be continued for a sufficiently long period as a prolongation of elimination half-life and redistribution of carvedilol from deeper compartments are to be expected. The duration of the supportive/antidote therapy depends on the severity of the overdosage. The supportive treatment should therefore be continued until the patient's condition has stabilised.

STORAGE CONDITIONS
Store below 30^{o}C

PACKAGING AVAILABLE
Carvedilol Tablets are available in Alu-Alu blister of 10 Tablets. Such blister strip containing 10 Tablets are packed in boxes of 10’s, 30’s and 100’s along with the pack insert. Not all presentations or all pack sizes may be marketed.

DATE OF REVISION OF PACKAGE INSERT
23/09/2015

NAME AND ADDRESS OF MANUFACTURER

TORRENT PHARMACEUTICALS LTD.
Indrad 382 721, District: Mehsana
INDIA.